

Policy on Safe Pharmacy Services and Medication Management

«Hospital Name»

Document control

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Prepared by	Medication Safety Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Pharmacy services and medication management at «Hospital Name» are governed by written guidance, overseen by a multi-disciplinary committee, and stay reliable even when the pharmacy is closed or stock runs out.

1. Purpose

This policy explains how «Hospital Name» runs its pharmacy services and medication management under written guidance, uses a multi-disciplinary committee to guide and update that guidance, keeps medications available when the pharmacy is closed or out of stock, and keeps staff informed of key changes.

This policy does not cover the hospital formulary, medication storage, prescription, dispensing, or reconciliation in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all pharmacy staff and clinical staff involved in medication management at «Hospital Name».

3. Policy standards

«Hospital Name» runs pharmacy services and medication management under a Medication Management Manual, guided by a multi-disciplinary committee that meets at least once

every three months and reviews the whole system at least once a year. There is a standard procedure to obtain medications when the pharmacy is closed or out of stock, and a process to keep relevant staff informed of shortages, recalls, and safety incidents.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

1. Do not manage medications without written guidance covering the formulary, procurement, storage, prescription, dispensing, administration, and monitoring — documented as a single Medication Management Manual, and supervised by a qualified individual across every patient care area.
2. Do not run the medication management committee without its defined roles, its required composition, or its terms of reference, and do not let it meet less often than once every three months, without documenting the minutes.
3. The Pharmacy and Therapeutics Committee shall conduct a documented review of the entire medication management system at least once each calendar year; no individual component or functional area may be excluded from that review.
4. Do not leave the hospital without a standard procedure to obtain medications when the pharmacy is closed or stock runs out.
5. Do not delay informing relevant staff of medication shortages, and do not take more than 24 hours to inform them of a drug recall.

5. What we do

5.1 Manage medications under written guidance

Written guidance — documented as a single Medication Management Manual — covers the formulary, procurement, storage, prescription, dispensing, administration, and monitoring of medications. A qualified individual supervises all pharmacy service activities, and this guidance applies across every patient care area in the organisation.

5.2 Guide medication management through a multi-disciplinary committee

A multi-disciplinary committee has defined roles and responsibilities for managing medications, in line with applicable legislation and regulations where relevant. Its responsibilities include, among other things, developing medication management processes, developing and revising the formulary, and evaluating medication use and safety incidents. The committee includes representatives from major clinical departments, administration, a pharmacist or clinical pharmacologist, the medication safety officer, nursing, and the Quality department. Its terms of reference — composition, meeting frequency, and quorum — are defined, and it meets at least once every three months, with minutes documented.

5.3 Review and update the medication management system

The committee reviews the whole medication management system at least once a year, covering rational use, medication errors, medication management processes, adverse drug reactions, patient safety, and high-risk medications.

«Hospital Name» designates a medication safety officer — this responsibility could be held by the patient safety officer. Related requirements are covered in the hospital's other policies.

5.4 Keep medications available when the pharmacy is closed or out of stock

A standard operating procedure sets out how to procure medications when the pharmacy is closed, or during a stock-out. It is preferable for «Hospital Name» to run a 24-hour pharmacy.

5.5 Keep staff informed of key changes

A process is in place to communicate medication shortages and stock-outs to clinicians and nurses. Staff are informed of a drug recall within 24 hours, and of any serious adverse events or patient safety incidents connected to medication use. The Pharmacy In-Charge, or a designated authority, is responsible for this communication, and it is documented. Related requirements are covered in the hospital's other policies.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring pharmacy services and medication management are resourced and implemented as required by this policy.
Medication Safety Officer	Owns day-to-day implementation of this policy; coordinates medication-safety processes; brings incidents and audit findings to the multi-disciplinary committee.
Multi-disciplinary committee	Guides the formulation and implementation of pharmacy services and medication management; reviews the whole system at least once a year; meets at least once every three months and documents minutes.
Pharmacy In-Charge / designated authority	Responsible for communicating medication shortages, recalls, and safety incidents to relevant staff; ensures the after-hours and stock-out procedure is operational.
Quality Coordinator	Audits this policy; holds training records and staff acknowledgements.

7. Quality monitoring

Monitoring area	What is monitored
Medication Management Manual	Current, complete, and supervised by a qualified individual across all patient care areas.
Multi-disciplinary committee	Constitution, terms of reference, required composition, meeting frequency of at least once every three months, and documented minutes.
Annual system review	Conducted by the committee at least once a year and covering all required topics (rational use, medication errors, ADRs, patient safety, high-risk medications).
After-hours / stock-out procedure	Standard operating procedure in place, tested, and used when required.
Staff communication	Drug recalls communicated within 24 hours; shortages and serious adverse events communicated and documented.

8. Training and staff acknowledgement

Pharmacy staff and clinical staff involved in medication management shall be familiar with the Medication Management Manual, the committee's role, the after-hours and stock-out procedure, and the staff communication process.

I have read the Policy on Safe Pharmacy Services and Medication Management of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to pharmacy staff, the multi-disciplinary committee, clinical staff involved in medication management, the Medication Safety Officer, the Pharmacy In-Charge, and the Quality Coordinator.

10. Abbreviations

Abbreviation	Meaning
ADR	Adverse drug reaction
DTC	Drug and Therapeutics Committee (the organisation's multi-disciplinary medication management committee)
MSO	Medication Safety Officer

NABH	National Accreditation Board for Hospitals and Healthcare Providers
MOM	Management of Medication (NABH Hospitals chapter)
SOP	Standard operating procedure

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
MOM.1.a	Commitment*	Sections 3 and 5.1 address written guidance documented as a single Medication Management Manual, qualified supervision, and coverage of all patient care areas.
MOM.1.b	Commitment	Sections 3 and 5.2 address the committee's defined roles, required composition, terms of reference, meeting frequency of at least once every three months, and documented minutes.
MOM.1.c	Achievement	Sections 3 and 5.3 address the committee's annual review of the whole medication management system covering all required topics.
MOM.1.d	Commitment*	Sections 3 and 5.4 address the standard operating procedure to obtain medications when the pharmacy is closed or during a stock-out.
MOM.1.e	Commitment	Sections 3 and 5.5 address the staff communication process for shortages, drug recalls (within 24 hours), and serious adverse events, with documented communication by the Pharmacy In-Charge or designated authority.

12. Required Records/Evidence Checklist

Medication Management Manual (MOM.1.a)

1. A single documented Medication Management Manual covering the formulary, procurement, storage, prescription, dispensing, administration, and monitoring.
2. Evidence that a qualified individual supervises all pharmacy service activities.
3. Evidence that the Manual applies across every patient care area in the organisation.

Multi-disciplinary committee (MOM.1.b)

1. Committee constitution record naming the required representatives: major clinical departments, administration, pharmacist or clinical pharmacologist, medication safety officer, nursing, and Quality department.
2. Written terms of reference defining composition, meeting frequency, and quorum.
3. Documented minutes of meetings held at least once every three months, naming decisions, owners, and due dates.

Annual system review (MOM.1.c)

1. Record of the committee's annual review of the medication management system.
2. Open-action tracking from the last review until closure.

After-hours and stock-out procedure (MOM.1.d)

1. Documented standard operating procedure to procure medications when the pharmacy is closed or during a stock-out.
2. Evidence the procedure has been tested and is operational.

Staff communication (MOM.1.e)

1. Records of communication of medication shortages and stock-outs to clinicians and nurses.
2. Records showing drug recalls communicated within 24 hours of notification.
3. Records of communication of serious adverse events and patient safety incidents connected to medication use.
4. Documentation identifying the Pharmacy In-Charge or designated authority as responsible for communication.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. MOM.1.

Disclaimer

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